Short-Term and Long-Term Health Risks of Alcohol and Drug Use

Category 01: Understanding Alcohol & Drugs | Topic 03 | Version 1.0 | 11 November 2026

Who is this for? Young people, adults, parents, educators and caregivers. This is educational information, not a diagnosis or personalized treatment plan. Publication status: editorial draft requiring clinical and safeguarding review.

What this lesson helps you understand

- Differentiate short-term harms, long-term health risks, dependence and withdrawal.
- Identify how alcohol and major drug groups can affect health, safety and daily life.
- Understand why risks differ by age, substance, circumstances and combinations.
- Recognize urgent warning signs and appropriate help-seeking actions.
- Practice applying reliable information through optional fictional scenarios.

Core reading is designed for roughly 12-15 minutes; optional activities add time. The estimate has not been user-tested. You can skip directly to the safety guidance or key takeaways.

01 | Understanding risk without fear or stigma

A health risk is the possibility that an action or exposure may cause harm. The presence of risk does not mean harm is inevitable, and the absence of immediate symptoms does not prove a substance is safe. Alcohol and other psychoactive substances can affect thinking, reaction time, breathing, blood pressure, sleep and organ function. Some effects emerge quickly; other problems develop after repeated exposure or over years. [1-4]

Different substances have different mechanisms. Risk is affected by dose, potency, frequency, route of exposure, age, underlying health, prescribed medicines, substance combinations and the safety of the surrounding environment. Unknown or contaminated products add uncertainty. Avoid comparing risks using a single label such as “hard” or “soft” drug. [2,4]

Essential distinction: A person can experience poisoning, an injury or another serious event without having a substance use disorder. Conversely, someone with a substance use disorder may have harms that are not immediately visible.

02 | Immediate and cumulative harm

Short-term (minutes to days)	Long-term (months to years)
Impaired decisions, attention and coordination; falls, crashes and injuries.	Disease of the liver, heart or other organs; increased risk of certain cancers from alcohol.
Poisoning, overdose, dangerous interactions and changes in breathing or consciousness.	Changes in memory and functioning; possible substance use disorder and associated health and social consequences.
Panic, agitation, confusion, or severe intoxication depending on the substance.	Persistent health problems and worsening daily functioning in some people; effects vary considerably by drug.

These columns describe timing, not severity. A short-term event can be fatal; long-term harms are not guaranteed. Both deserve attention. [1-4]

03 | Alcohol: what may happen soon after drinking?

Alcohol contains ethanol, a toxic psychoactive substance. It can impair judgment, memory formation, reaction time, balance and coordination, even while a person feels confident. Some people become drowsy, impulsive, nauseated or confused. Greater exposure increases the likelihood of poisoning; severe poisoning may suppress breathing and consciousness. [1,3]

Injuries and other immediate consequences

Alcohol can increase the likelihood of road crashes, falls, drowning and other accidents. It may contribute to unsafe decisions and interpersonal harm. Drinking in a situation involving driving, riding a motorcycle, operating machinery, supervising children or swimming introduces particular hazards. Risk exists before a person develops dependence. [1,3]

Alcohol and other substances together

Combining alcohol with opioids or sedating medicines can intensify suppression of alertness and breathing. A clinician or pharmacist should advise on interactions with prescribed medicines. Never use someone else’s prescription or assume that mixing substances is safe. [3,6]

04 | Alcohol: risks from repeated consumption

Over time, alcohol can damage the liver, contribute to hypertension and heart disease, affect the pancreas and digestive system, and increase the likelihood of alcohol use disorder. Alcohol has a causal link with several cancers, including cancers of the breast, liver, mouth/throat, oesophagus and bowel. Cancer risk may rise even with relatively low alcohol intake; drinking more generally produces greater risk. [1,5]

Alcohol may also worsen sleep and emotional difficulties. These connections are complex: anxiety or depression may contribute to substance use, and alcohol may worsen symptoms or interfere with care. Professional assessment should consider both. [1,3]

05 | Drug-specific risks: understand the differences

Substance group	Possible immediate problems	Possible longer-term concerns
Opioids	Sleepiness and slowed breathing; overdose can be fatal.	Dependence and overdose risk, especially with unregulated products or sedative combinations.
Stimulants	Agitation, elevated heart rate, overheating, high blood pressure.	Cardiovascular, sleep and mental-health difficulties; stimulant use disorder may develop.
Sedatives	Drowsiness, poor coordination and potentially dangerous interactions.	Dependence and hazardous withdrawal with some medicines; requires clinical supervision.
Cannabis	Impaired attention, memory, coordination; possible anxiety or paranoia.	Cannabis use disorder; in some people, cognitive and mental-health concerns, especially with frequent early use.
Inhalants	Unpredictable toxic effects; fainting, abnormal heartbeat or sudden death.	Potential neurological, liver or kidney damage with some exposures.

The table is a simplified educational comparison, not a method to predict an individual outcome. Drug strength, contaminants, medical conditions, age and other exposures matter. Prescribed medicines have legitimate uses when taken as directed. [2,4,6,7]

06 | Why young people require special care

Adolescence is a period of continuing brain and social development. Alcohol and other drug use during these years can impair judgment and safety, interfere with learning, and is associated with a greater risk of later substance-related problems. Frequent or heavy cannabis use in adolescence is associated with problems in memory, attention and functioning. Effects differ between individuals and do not justify labeling any student as “damaged.” [4,7]

Prevention should emphasize accurate information, supportive adults, school belonging, skill-building and confidential, age-appropriate help. Young people should never be required to reveal personal experiences to participate in educational activities.

07 | Additional situations that need care

Pregnancy and breastfeeding

Alcohol exposure during pregnancy can harm fetal development. People who are pregnant or planning pregnancy should seek professional advice about alcohol and other substances, including prescribed medicines, rather than changing treatment without guidance. [1,3]

Older adults and people with medical conditions

Certain health conditions, medications, changes in metabolism and fall risks can make exposure to alcohol or drugs especially hazardous. Medicine and substance interaction advice should be individualized by qualified healthcare professionals.

Using alcohol to manage stress or sleep

Someone may notice temporary relaxation, yet alcohol can disrupt sleep and make future stress or mood difficulties harder to manage. A short-term feeling of relief is not proof that a substance is treating the underlying problem. [1,3]

Unknown products and unexpected potency

Unregulated products may contain unexpected ingredients or strengths. Severe symptoms require urgent care; an online lesson cannot determine product contents or tell someone that an unknown substance is safe.

08 | Understanding use, harm, dependence and withdrawal

Use: consumption of a substance, which alone does not constitute a diagnosis. Harmful or risky patterns: exposure that causes or increases the chance of physical, psychological, social or safety problems. Substance use disorder: a clinically assessed pattern of problematic substance use and impaired functioning. Dependence and withdrawal: physiological and behavioral processes that may occur in some individuals and with some substances. These are related but not interchangeable terms. [2,8]

Withdrawal caution: A person who has developed physical dependence on alcohol or certain sedating medications may become seriously ill when stopping abruptly. Severe alcohol withdrawal can cause seizures or delirium. A qualified clinician should assess withdrawal risk; this chapter does not give a home detoxification plan. [8]

09 | Separating facts from common misconceptions

Myth	Better explanation
“Only addicted people are harmed.”	A single exposure can cause injury or poisoning without dependence.
“Beer is safer because it is common.”	Beer contains ethanol; amount consumed matters, and no alcoholic drink is risk-free.
“A prescription medicine is safe to share.”	Prescriptions are individualized; sharing or combining medicines can be dangerous.
“Everyone who uses drugs becomes addicted.”	Not everyone develops a disorder, but serious risks may still occur.
“A person who looks awake can drive safely.”	Appearance and confidence do not reliably indicate unimpaired judgment or reaction time.

10 | Fictional situations: learn from everyday life

Scenario A — A school social event

A 16-year-old student, Aline, hears classmates say that one alcoholic drink cannot harm anyone. They want her to join them. Aline can choose not to drink and seek help from a trusted adult if pressure continues. Learning: peer pressure can distort how people judge risk; adolescents need support rather than shame.

Scenario B — A worker riding home

After a celebration, Jean feels confident and proposes riding his motorcycle home. Friends notice he has been drinking. They suggest safe transport. Learning: alcohol-related impairment is not reliably measured by self-confidence or a short rest.

Scenario C — Medicines at home

Mireille has trouble sleeping. A relative offers a strong sleeping tablet prescribed for someone else. She declines and asks a healthcare professional about her symptoms. Learning: prescribed sedatives can have serious risks and should not be shared.

Scenario D — A family member seems unwell

A parent notices a young adult missing work, sleeping poorly and seeming unwell. Rather than assuming addiction, they ask about wellbeing and offer support finding a clinician. Learning: concerning changes have many possible causes; listen without diagnosing.

Scenario E — Breathing changes at a gathering

A person becomes difficult to wake and has unusually slow breathing after taking an unknown substance. A bystander immediately seeks emergency medical assistance and stays with the person. Learning: an unresponsive person with abnormal breathing may be experiencing a life-threatening emergency.

11 | Optional interactive activities

These activities describe future tap-to-answer interactions. In this PDF, read the options, choose mentally, then compare with the feedback. No personal substance-use history is requested. Any question may be skipped.

Activity 1 — Identify the kind of risk

Question: Someone who drank alcohol has trouble coordinating movements and plans to ride home. Is the main immediate concern (A) an impaired-driving injury risk, (B) guaranteed future dependence, or (C) no risk if the person feels awake?

Feedback: A is best. Alcohol can impair movement and judgment immediately. This alone cannot predict who will develop dependence. [1,3]

Activity 2 — Long-term health

Question: Which statement is accurate? (A) Alcohol causes cancer only after addiction, (B) Alcohol increases risk of certain cancers even without an alcohol use disorder, or (C) only spirits carry cancer risk.

Feedback: B is accurate. The carcinogenic exposure is ethanol in alcoholic beverages. Risk is not restricted to people diagnosed with alcohol use disorder. [1,5]

12 | More optional interactive learning

Activity 3 — Medicines and combinations

Fictional choice: A friend offers an unprescribed sedative after someone has been drinking. Choose: (A) Accept because it is medication; (B) Refuse, avoid mixing, and obtain medical advice if needed.

Feedback: B is safer. Alcohol with sedating substances may dangerously increase impairment and breathing suppression. [3,6]

Activity 4 — Possible overdose

Fictional choice: Someone is unconscious with slow breathing. Choose: (A) Let them sleep alone; (B) Seek emergency medical help immediately, stay with them, and follow responder instructions.

Feedback: B is safer. Do not assume someone with abnormal breathing is merely asleep. [6]

Activity 5 — Compassionate support

Fictional choice: A student has become withdrawn and is missing lessons. Choose: (A) Publicly accuse them of addiction; (B) Speak privately, avoid assumptions, and seek appropriate support if there are concerns.

Feedback: B is more appropriate. Changes in behavior may have many causes and the young person deserves privacy and safeguarding. [4]

13 | How interactive feedback should work on the website

Element	Educational and accessibility requirement
Prompt	One plain-language question with an optional skip link.
Options	Clearly labeled, keyboard-operable controls with large touch targets.
Feedback	Immediate, nonjudgmental explanation for each option; allow retry.
Privacy	Do not request or save disclosure of personal substance use in ordinary lessons.
Alternatives	Text-equivalent content for images; captions/transcripts for optional media.
Help	Get Help link remains available without finishing a quiz or video.

These are specifications for a future interactive website; the PDF itself is a readable educational artifact, not a functioning screening or diagnostic tool.

14 | Discussion activity for families and educators

Use the fictional Scenario B or C, then ask: What safety issue should be noticed first? What assumption would be unhelpful? Who could provide appropriate advice? Which respectful statement could start a conversation? Never require students or family members to disclose their own substance use in a group.

15 | When to seek urgent medical attention

Immediate danger: Seek emergency medical assistance for an unresponsive person, breathing that is unusually slow or irregular, seizures, severe confusion, collapse or other serious poisoning signs. Stay with the person when safe, follow emergency responder instructions, and do not wait for every symptom to appear. If available and an opioid overdose is suspected, responders may advise naloxone; medical help is still required. [6]

Do not encourage vomiting, give coffee as a “cure,” or leave someone unconscious to sleep it off. If a person is unconscious but breathing, follow local emergency guidance for safe positioning and continued observation until help arrives. If they are not breathing normally, follow emergency dispatcher directions for resuscitation. [6]

Seeking help before an emergency

Someone concerned about alcohol or drug use can request a confidential health assessment before a crisis occurs. Encourage treatment without blame. People considering stopping alcohol after long periods of heavy use should discuss withdrawal risks with a clinician first. [8]

Rwanda-specific implementation note

Rwanda Ministry of Health Health Sector Strategic Plan V (2024/25-2028/29) identifies alcohol consumption among significant noncommunicable disease risk factors and recognizes gaps in awareness and care. [9] This supports locally relevant education but does not constitute official endorsement or accreditation of this lesson. Before publication, a Rwanda-based reviewer should verify referral locations, appropriate emergency numbers and age-specific safeguarding requirements.

16 | Key takeaways

- Short-term harms can occur after a single episode; they do not require dependence.
- Long-term exposure increases the risk of some chronic diseases; alcohol causes several cancers.
- Opioids, stimulants, sedatives, cannabis and other drugs have different risk profiles.
- Age, health conditions, medication interactions and mixing substances can change risk.
- Clinical assessment is needed to identify a substance use disorder or plan withdrawal care.
- Unconsciousness, abnormal breathing and seizures require urgent medical attention.
- Accurate, non-stigmatizing education and early support help protect people and communities.

17 | Knowledge check: answers and learning goals

Question	Model answer
Can injury happen without addiction?	Yes. Impairment, poisoning or accidents may occur without a substance use disorder.
Do all drugs have identical effects?	No. Their mechanisms, acute dangers and longer-term risks differ.
Does every person who drinks develop liver disease?	No. Risks vary; not everyone develops a disease, and serious risk still warrants prevention.
Is quitting heavy alcohol use always safe without advice?	No. Physically dependent people can experience dangerous withdrawal.
What matters most in a possible overdose?	Prompt emergency help and attention to breathing and consciousness.

18 | Glossary

Acute harm: A health or safety problem occurring soon after exposure.

Chronic risk: A likelihood of health problems from longer-term exposure.

Overdose or poisoning: A dangerous reaction to substance exposure requiring medical attention.

Intoxication: Changes in thinking, behavior or physical functioning after exposure.

Substance use disorder: A clinically assessed condition involving problematic substance use.

Withdrawal: Symptoms arising after reducing or stopping exposure to certain substances following dependence.

Carcinogen: An exposure that can cause cancer.

Safeguarding: Measures to protect children or others at risk from harm.

19 | Publication review and evidence handling

• Clinical reviewer: verify organ-system claims, substance categories, overdose guidance, alcohol withdrawal and medicine interaction language.

• Child safeguarding reviewer: verify age-appropriate scenarios, school guidance and confidentiality boundaries.

• Rwanda legal and service reviewer: verify local referral pathways and any legal or policy claims.

• Accessibility and language reviewers: test Kinyarwanda/French terminology, screen reader content, plain language and low-data alternatives.

• Content owner: record reviewer, approval date, revision date and sources used. This draft is not yet approved for public clinical use.

20 | Source and evidence register

[1] WHO. Alcohol (28 June 2024).

https://www.who.int/news-room/fact-sheets/detail/alcohol

[2] WHO. Drugs (psychoactive) - health topic.

https://www.who.int/health-topics/drugs-psychoactive/

[3] CDC. Alcohol Use and Your Health (14 Jan 2025).

https://www.cdc.gov/alcohol/about-alcohol-use/

[4] NIH. Youth and Young Adult Alcohol and Other Substance Use.

https://health.nih.gov/health-topics-a-z/youth-drug-use

[5] National Cancer Institute. Alcohol and Cancer Risk.

https://www.cancer.gov/about-cancer/causes-prevention/risk/alcohol/alcohol-fact-sheet

[6] CDC. Preventing Opioid Overdose.

https://www.cdc.gov/overdose-prevention/prevention/

[7] CDC. Cannabis Health Effects.

https://www.cdc.gov/cannabis/health-effects/

[8] NIAAA. Alcohol Use Disorder: From Risk to Diagnosis to Recovery.

https://www.niaaa.nih.gov/health-professionals-communities/core-resource-on-alcohol/alcohol-use-disorder-risk-diagnosis-recovery

[9] Rwanda Ministry of Health. Health Sector Strategic Plan V (2024/25-2028/29).

https://www.moh.gov.rw/strategic-plans-priorities

[10] NIAAA. Alcohol and the Human Body (updated March 2026).

https://www.niaaa.nih.gov/alcohols-effects-health/alcohol-topics-z/alcohol-facts-and-statistics/alcohol-and-human-body

Editorial note: Sources were consulted for scientific framing. Exact page dates, clinical recommendations and Rwanda-specific service information should be checked again during professional review. A PDF is a downloadable learning resource; the future website should implement the interactive activities as accessible controls.

End of Topic 03 | Suggested next topic: How risky use can develop into dependence and a substance use disorder.